



Nutrition and Hydration Policy

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	All disciplines

2. Purpose

The purpose of this policy is to set out the framework for a strategic and co-ordinated approach to nutrition from admission to discharge or transfer of care across Armada Kalamunda Group. This will ensure that all patients have access to appropriate nutrition and hydration which is essential to support general well-being, recovery from illness or surgery and to prevent malnutrition.

3. Policy statement

In line with the 'The Australian Council on Healthcare Standards' requirements this policy will look at the following:

- Governance, risk screening, assessment, care planning, monitoring and discharge planning.
- Nutrition requirements of the patient, planning and provision of food, fluids and nutrition supplements.
- Mealtime environment and assistance to eat and drink.
- Patient information and communication, nutrition education and training for staff across AKG.

3.1 Nutrition and Hydration Screening

3.1.1 General Screening

On admission patients should have their nutrition and hydration status screened and documented by nursing or medical staff. Including:

- Weight and height (as appropriate)
- The safest route of providing nutrition and hydration e.g. oral, enteral, intra-venous.
- The amount of assistance required to maintain nutrition and hydration
- Risk of aspiration - refer to [AKG Dysphagia Screening and Assessment Procedure](#) and speech pathology.
- Specific dietary needs should be documented and communicated to ensure appropriate delivery of nutrition requirements.
- The last time the patient was able to consume a meal and/or received nutrition support
 - Patients are at risk of developing refeeding syndrome if they have received very little or no nutrition for more than 5 days, refer to [RPMG Refeeding Syndrome Clinical Guideline](#). If the patient requires enteral feeding, refer to [AKG Enteral Feeding and Enteral Tubes Management – Adult](#) Appendix 2 Hospital Wide After-Hours Enteral Feeding Regime – refeeding syndrome protocol.

3.1.2 Malnutrition Screening

- All adult inpatients are to be screened for risk of malnutrition within 8 hours of admission using the Malnutrition Screening Tool (MST) located within the Skin and Pressure Injury Assessment tool (AKMR121.9) as part of the [AKG Pressure Injury Prevention and Management Policy](#).
- Adult inpatients are to be rescreened using the MST weekly, with any change in condition and episode of care.
- Paediatric inpatients will be screened on admission using the Paediatric Nutrition Screening Tool (PNST) as part of:
 - AKMR 115.0.3 Comprehensive Admission Screening – short stay
 - AKMR 115.0.1 Comprehensive admission and screening – 12 years and over
 - AKMR 115.0 Comprehensive Admission and Screening – 0-12yrs
- In addition, paediatric inpatients are screened daily for inadequate nutrition using the AKG Paediatric Skin Assessment, Pressure Injury and Falls Risk Assessment & Management form.
- For patients at risk of malnutrition (MST ≥ 3 adults or PNST ≥ 2 paediatrics):
 - Refer to a Dietitian
 - Commence food and fluid intake chart to establish nutritional intake [Appendix 1 Food Intake Monitoring](#)
 - Document a nutrition and hydration plan in the patient health record and in the relevant care plan.

3.2 Nutrition Assessment

3.2.1 General Assessment

- Nursing, medical and allied health staff should routinely observe a patient's nutrition and hydration status (for example oral hygiene, teeth/dentures, ability to swallow effectively, ability to open packages and self-feed, special dietary requirements).
- Nursing, medical and allied health staff are to refer to dietetics for further nutrition assessment if indicated.
- Indications for further nutrition assessment include but are not limited to:

• Recent unintentional weight loss (>5% in 6 months)	• Renal – pre-dialysis, haemodialysis – poorly controlled
• Underweight, body mass index (BMI) <18kg/m ² for <65 years or <24kg/m ² for >65 years	• Constipation and/or diarrhoea
• Patients with/at risk of malnutrition (MST ≥ 3)	• Acute flare up of diverticular disease, ulcerative colitis or Crohn's disease
• Enteral or Total Parenteral Nutrition support	• Diabetes (newly diagnosed or poorly controlled T1 or T2 diabetes mellitus)
• Patients at risk of refeeding syndrome	• Surgical – gastro-intestinal surgery, pancreatitis
• Eating disorders	• Cardiovascular disease

3.2.2 Dietetic Assessment

- Dietetic assessment includes agreed methods to assess nutrition and hydration and establish a nutritional diagnosis.
- Methods include, but are not limited to assessing:
 - Current and previous nutrition and hydration status
 - Nutrition related physical exam e.g. Subjective Global Assessment (SGA)
 - Nutrition impact symptoms, nausea, vomiting, poor appetite, weight history, bowel function, fat and muscle wasting
 - Refer to [AKG Dietetics Adult Malnutrition Diagnosis and Documentation Guideline](#)
- Refer to [AKG Dietetic Inpatient Prioritisation and Assessment Guideline](#) for details on dietetic caseload priorities.

3.3 Provision of Nutrition and Hydration

All patients are to receive adequate nutrition and hydration according to individual requirements; this may be delivered via oral, enteral or parenteral routes.

AKG is responsible for providing food and fluids that comply with the Nutrition Standards for adult inpatients in WA Hospitals. Refer to [EMHS Nutrition Guidelines for Adult Inpatients in EMHS Hospitals](#). Culturally appropriate options should be available as required for all patients/consumers.

Food and Patient Service Assistants that deliver patient meal trays receive training to check the name on the menu slip against details on the iSOFT Clinical Manager ward list and then check the name above the patient's bed before delivering food and drink to patients. Alternatively, if no name is listed above the patient's bed, the patient will be asked to state their name for verification.

The [AKG Ward Nutrition Support Protocol \(Appendix 2\)](#) outlines Nursing initiated commencement of oral or enteral nutrition support outside of Dietitian core business hours or during times of high clinical dietetic demand/dietetic staffing shortages.

3.3.1 Hydration

- Fluids are to be readily available, accessible and provided with assistance (as required) at all times in the patient's room, unless fluid restricted.
- Patient's requiring thickened fluids are at increased risk of dehydration.
- Fluid may be provided via oral, intravenous/parenteral, subcutaneous or enteral routes as clinically indicated.
- Patient's hydration status must be clearly documented in the patient health record.
- For patient's requiring a fluid restriction (for medical reasons) this is to be clearly documented in the patient health record.
- Monitor fluid balance as clinically indicated and document on the *Daily Intake/Output Record AKMR144*.
- Accurate fluid balance documentation is essential to the appropriate management or treatment of fluid imbalance, to assist with determining fluid volume intake refer to [Appendix 3 Fluid volume measurements](#).

3.3.2 Oral Nutrition

- All patients able to consume oral nutrition should receive a full meal service every day.
- All patients should receive a menu and where feasible be supported to complete their own menu.
- Specific patient groups will be provided with a diet appropriate to their needs. These may include medically diagnosed food allergies and therapeutic diets e.g. low potassium, gluten free, low fibre. Refer to [AKG Therapeutic Diet and Fluids](#) for further details on specific diet and fluids.
- Patients with swallowing difficulties are to be referred to speech pathology.
- Patients on modified consistency diets and thickened fluids are at higher risk of inadequate oral intake, especially if unable to self-feed.
- Oral nutrition support is initiated by the dietitian and documented on the Nutrition Supplement Chart (AKMR60.1) including the nutrition product name, volume, frequency and time to be delivered.

3.3.3 Artificial Nutrition

- If oral nutrition is inadequate and/or contraindicated patients should receive artificial nutrition support (enteral and/or parenteral nutrition) to meet their nutrition requirements.
- Patients who are nil by mouth for more than three days are to be considered for artificial nutrition support.
- Enteral nutrition is used to provide nutrition and hydration to patient who cannot safely consume adequate nutrition and/or hydration orally.
- Refer to [AKG Enteral Feeding and Enteral Tubes Management – Adult](#) for specific details on enteral feeding methods
- Parenteral nutrition can be used for patients who are unable to tolerate or absorb adequate nutrition via oral or enteral routes.
- Refer to [AKG Total Parenteral Nutrition](#) for specific details on parenteral feeding methods.

3.3.4 Mealtime Environment

- Disruptions at mealtimes may reduce a patient's nutritional intake, all staff are to ensure that the environment is safe and conducive to eating by:
 - Minimising non-essential therapy or clinical procedures
 - Adjusting surroundings, sights, smells, noise and lighting
- The following aspects of patient care should be considered prior to meal delivery:
 - Waking the patient
 - Ensuring the patient is correctly positioned for eating
 - Clearing bedside tables
 - Toileting
 - Hand washing
 - Ensuring dentures are in place and mouth care is attended
- Appropriate regular assistance is to be provided to patients that are unable to take fluids and diet independently. Red mats are utilised to highlight patients who may require assistance and/supervision with oral intake. Refer to [AKG Red Mat System Guideline](#) for details.

- Refer patients who are identified as requiring specialised equipment to eat and drink to occupational therapy.
- Refer to [Appendix 4 Feeding Behaviours and Dementia](#) for specific strategies for patients with dementia feeding behaviours.

3.3.5 Management of Food Allergies and Intolerances

- A food allergy is an abnormal immune mediated reaction to ingested food, resulting in clinical symptoms. Reactions can occur after eating a small amount of food and be life threatening, such as anaphylaxis. It is the responsibility of the treating medical team to ensure a clinical alert is recorded for patients with severe food allergies. See [AKG Adverse Drug Reaction Clinical Alert Procedure](#) and [WA Clinical Alert \(MedAlert\) Policy](#). A food intolerance does not involve the immune system and does not cause severe life-threatening allergic reactions and should not be listed as a clinical alert.
- On admission, nursing is to identify potential food allergies or need for therapeutic diets from patients, their next of kin or facility handover. Any food allergy or intolerance reported should be documented on the Nursing Admission Form and must be immediately entered into the iSOFT Clinical Manager (iCM) system under the 'Diet' and 'Diet Notes' sections of the Handover tab; with verbal notification to the kitchen staff to make them aware.

3.4 Nutrition Care Planning

3.4.1 Patient Education

- Information regarding nutrition and hydration intervention should be communicated to patients and their carers in formats that they can understand and documented in the patient health record.
- Information should be tailored to the patient's individual nutrition care plan where indicated.
- Information should be available in languages/formats other than English for culturally and linguistically diverse (CALD) populations.
- The use of interpreters should be considered to increase communication and understanding.
- Patients are informed about hospital food services, mealtimes and policies involving food or drink brought in from home or the community where appropriate. Refer to Armadale Health Service Information for patients and their carers, families and visitors brochure and [Appendix 5 Bringing Food From Home information leaflet](#).
- Feedback and progress should be provided to the patient/carers by the relevant staff at appropriate intervals throughout the patient journey.

3.4.2 Monitoring

- An individualised management plan is to be documented in the patients' health records as soon as practical.

3.5 Nutrition Specific Monitoring

- The following clinical indicators of nutrition and hydration status should be monitored regularly by nursing/medical/allied health staff as part of the patient/consumer's individual nutrition and hydration care plan:

Clinical indicator	Frequency of monitoring
• Weight	On admission and at least once a week. Daily if there are concerns regarding fluid balance
• Height	On admission
• Nutrition intake	Daily until stable, then twice weekly or as clinically indicated
• Fluid balance	Daily until stable then as clinically indicated
• Biochemistry	Twice weekly initially until stable or as clinically indicated
• Bowels	Daily initially then reducing to twice weekly as indicated
• Urine output	At least daily
• Temperature	Daily or as clinically indicated

-
- Enteral nutrition and tubes should be monitored according to [AKG Enteral Feeding and Enteral Tubes Management – Adult](#)
- Parenteral nutrition should be monitored according to [AKG Total Parenteral Nutrition](#)
- Dietitians and medical officers may request additional monitoring measures depending on the individual patients' nutrition and hydration care plan, this should be documented in the patients' health record.
- The short- and long-term goals of nutrition support should also be monitored as clinically indicated.

3.5.1 Discharge Planning

- All patients where required should be discharged with a nutrition and hydration care plan, including details of appropriate follow up.
- The following factors should be considered in the patient discharge planning:
- The discharge destination.
 - Continuity of the patient's nutrition and hydration care plan.
 - Access to nutrition support and hydration whether oral or enteral as per the [AKG Registration and Recording Instructions for Home Enteral Nutrition Self Care](#).
 - Access to support services (i.e. assistance with feeding or meal preparation).
 - Allied health recommendations and care plans.
 - Appropriate follow up in the outpatient/community setting to monitor nutrition support.
 - Consumer information and education.
 - Documentation and handover of the care plan to follow-up services.

3.6 Patient Information and Communication

- Patient information, education material and communication around nutrition should be from evidenced based academic sources.
- The Corporate Communications team should also be consulted for all locally developed patient resources.

- Patients and/or consumers should be involved to ensure education and communication materials are culturally and linguistically appropriate.
- Patient Education Information regarding the role of nutrition and hydration in promoting recovery and general wellbeing should be made available to all patients, as clinically relevant or requested.
- Patients/Carers should be:
 - involved in their nutrition care plans.
 - given menu choices and information about meal services to enable appropriate choices.
 - given education to ensure a good understanding of nutrition and hydration interventions.
 - provided with regular feedback about their progress.
 - provided with education and information regarding safe artificial nutrition support strategies to treat or prevent malnutrition if required.
 - given education around risk of choking or aspiration for those who continue to consume foods and fluids that put them at risk.
- Information should be:
 - safe, accurate and evidence based.
 - displayed and readily accessible by all patients.
 - written and formatted to be easily understood by the general public.
 - available and understood by culturally and linguistically diverse populations that access the service.
 - conducive to promoting health literacy.
- Staff should utilise interpreter services to promote effective communication with culturally and linguistically diverse populations.

3.7 Education and Training for Staff

- Training and education programs enhance an understanding of the link between good nutrition care, preventing malnutrition and delivering better patient outcomes.
- All staff involved in nutrition care should:
 - understand their role and responsibilities and receive appropriate education and training.
 - be aware of the role of food and nutrition in preventing malnutrition, maximising patients' clinical outcomes and quality of life.
 - be appropriately qualified and have knowledge of key aspects of nutrition care.
 - be aware that nutrition status may be compromised in patients who are overweight or obese.

4. Roles and responsibilities

Role	Responsibility
Dietitian	Nutrition screening, assessment for nutrition and hydration, monitoring and intervention including patient education, and discharge planning. Ensuring the patient environment is safe and conducive to eating and the patient is provided with a diet appropriate to their needs. Providing nutrition education and training for staff.

Role	Responsibility
Nurse	Nutrition and hydration screening. Participating in relevant nutrition education and training for staff. Ensuring the patient environment is safe and conducive to eating and the patient is provided with a diet appropriate to their needs.

5. Legislative context

The following documents are required to give effect to this policy:

- [EMHS Food and Nutrition Policy](#)

6. Supporting information

The following documents support the implementation of this policy:

- [AKG Dysphagia Screening and Assessment Procedure](#)
- [RPBG Refeeding Syndrome Clinical Guideline](#)
- [AKG Dietetic Inpatient Prioritisation and Assessment Guideline](#)
- [AKG Therapeutic Diet and Fluids](#)
- [AKG Enteral Feeding and Enteral Tubes Management – Adult](#)
- [AKG Total Parenteral Nutrition](#)
- [AKG Red Mat System Guideline](#)
- [AKG Dietetics Prescribing High Energy High Protein Diets Guideline](#)
- [WA Health Nutrition Standards Guidelines](#)
- [EMHS Nutrition Guidelines for Adult Inpatients in EMHS Hospitals](#)
- [AKG Registration and Recording Instructions for Home Enteral Nutrition Self Care](#)
- [AKG Pressure Injury Prevention and Management Procedure](#)
- [AKG Adverse Drug Reaction Clinical Alert Procedure](#)
- [WA Clinical Alert \(MedAlert\) Policy](#)

7. Compliance, monitoring and evaluation

Compliance against this policy will be evaluated with:

- Audit of nutrition screening – ongoing CoBRA screening
- Catering patient food satisfaction surveys - quarterly
- Compliments and complaints relating to food service and nutrition - ongoing
- Quarterly meal audits
- Annual Nutrition Standards audits - annual
- Dietetic documentation audits

Compliance monitoring will be conducted and recorded by the Nutrition and Dietetics Department, Nursing, Catering and Safety and Quality and tabled at Nutrition Management Sub-committee.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5: Action 5.1 Clinicians use the safety and quality systems from the Clinical Governance Standard.
- Standard 5: Action 5.28 The workforce uses the systems for preparation and distribution of food and fluids.

9. References

1. Ensell C, Matheson N. (2009). Mealtime behaviours in people with dementia in the absence of dysphagia: education of nursing staff in an acute care setting. *ACQuiring Knowledge in Speech Language and Hearing*, 11(2), 92-96

10. Glossary

The following definitions are relevant to this policy.

Term	Definition
Malnutrition	Malnutrition is defined as 'a state of nutrition in which a deficiency or excess (imbalance) of energy, protein, and other nutrients causes measurable adverse effects on tissue/body form (body shape, size and composition), function and clinical outcome'.
Nutrition Screening	A rapid, simple and general procedure used by nursing, medical or other staff. Often done at first contact with the patient, to detect those who have significant nutritional problems or significant risks of such problems. With clear guidelines for action e.g. simple dietary intervention measures or referral to a Dietitian for a comprehensive nutrition assessment and intervention.
Nutrition Assessment	Is a comprehensive approach to gathering pertinent data in order to evaluate nutritional status and identify nutrition-related problems, completed by a Dietitian? The assessment often includes patient history, medical diagnosis and treatment plan; nutrition and medication histories, nutrition related physical examination including anthropometry, nutritional biochemistry, psychological, social and environmental aspects.
Nutrition Support	The provision of nutrients to patients orally and/or enterally (administration into the stomach or intestine) and/or parenterally (intravenous infusion) for the purpose of improving or maintaining a patient's nutrition status.

11. Policy contact

Enquiries relating to this policy may be directed to:

Title: Coordinator Dietetics and Nutrition
 Department: Nutrition and Dietetics
 Email: linda.freeman@health.wa.gov.au

12. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.0	25/08/2025	25/08/2028	Appendix 2: Removed medical team charting on medication chart. Nurses instead to use pre-filled Nutritional Supplements and Products forms.

13. Authorisation

This policy has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray, Director Allied Health and Ambulatory Care
Authorisation date	20/08/2025
Published date	25/08/2025

Appendix 1 Food Intake Monitoring

Indications

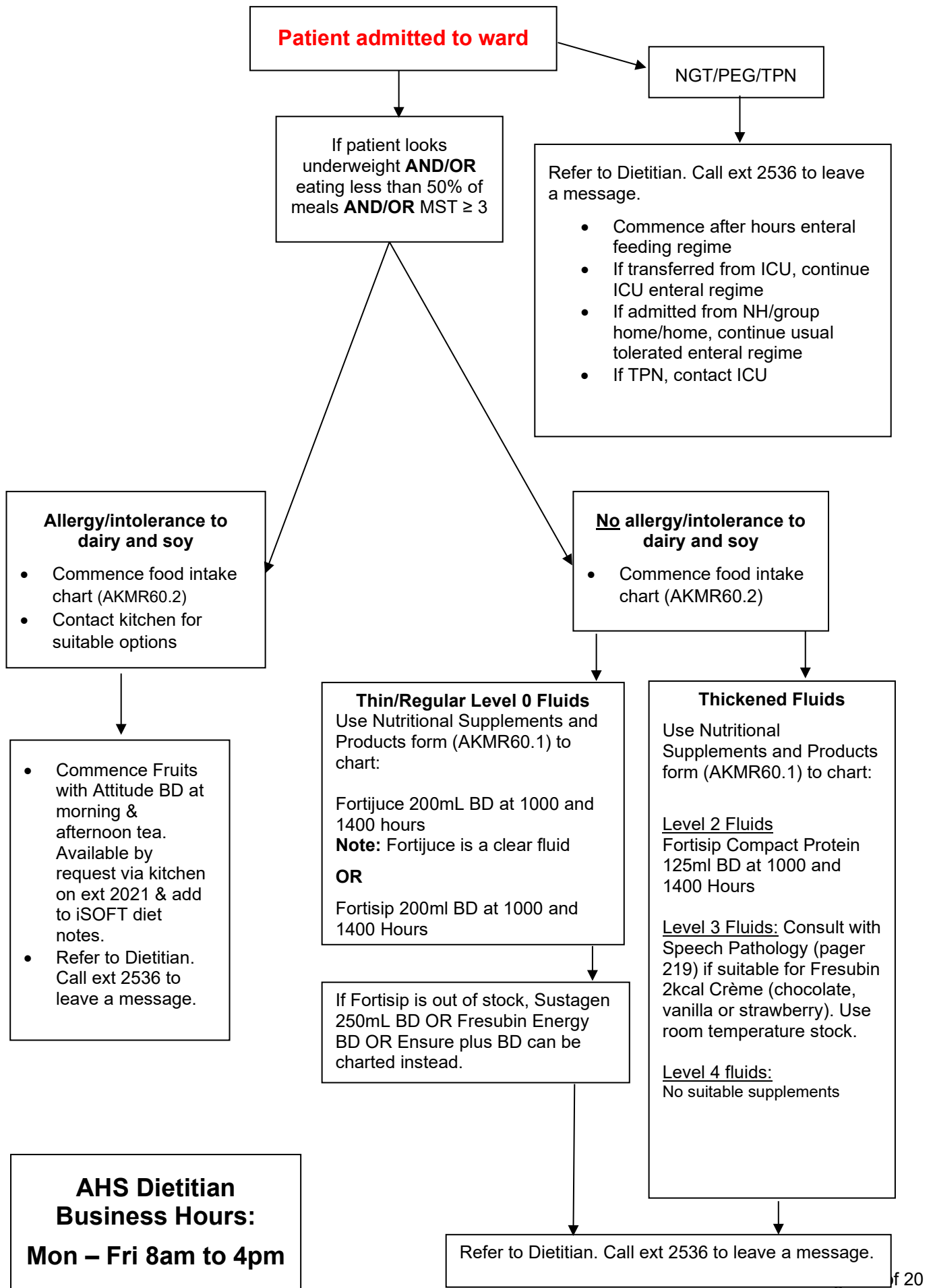
Method of nutrition screening by Medical Officer, Nursing Staff, Dietitian for patients including, but not limited to:

- Suspected or observed decreased oral intake
- Diabetes
- Neither nursing staff nor patient is unable to report the number of meals/snacks consumed
- Confirming oral nutrition supplement /snack compliance
- Patients on transitional enteral feeding regime

Procedure

- Indication for food intake monitoring is identified by Medical Officer, Nursing Staff, Dietitian or other staff member.
- Food Intake Chart (AKMR 60.2) should be documented in the Nursing Care Plan and placed in patient's nursing/bedside chart for completion of three (3) days of intake and document in the patient health record.
- Establish if there is a reason the patient has had suboptimal intake (e.g. fatigue, anxiety, lack of appetite, dislike of meal) and document in the patient health record.
- **Do not** document food intake on the Food Intake Chart **until it has been consumed.**
- If a Food Intake Chart is commenced by Medical Officer or Nursing Staff and food intake is 50% or less consistently over the three days, then a referral to the Dietitian is to be generated via the patient journey board (PJB) or page and document in the patient health record .
- If a Food Intake Chart is commenced by Medical Officer or Nursing staff and food intake is greater than 50 to 100% consistently over the three days, then a referral to the Dietitian is not required (unless another indication for a Dietitian review is evident e.g. Malnutrition Screening Tool score, requires diet education) and document in the patient health record.

Appendix 2 Ward Nutrition Support Protocol



Appendix 3 Fluid Volume Measurements

Volumes reflect a fill level to within 2cm of vessel rim

Plastic white cup 130 mL



Sustain paper cup 230 mL



Plastic cup clear 150 mL



Plastic beaker cup 250 mL



Plastic jug 1000 mL



Tea cup 165 mL



Blue mug 200 mL



Brownes yoghurt 100 mL



Flavour creations custard 115 mL



Bulla ice cream 100 mL



Jelly cup 100 mL



Fruche 150 mL



Soup bowl 200 mL



Cereal/custard bowl 170 mL



Maroon jelly bowl 125 mL



Appendix 4 Feeding Behaviours and Dementia

The following table is a guide only. Advice should be sought from Dietitian or Medical Staff for individual patient management.

Mealtime behaviours and strategies	
Holding food or drink in the mouth	Provide verbal cues to chew and swallow Bring an empty spoon to the person's mouth to remind them to swallow the prior mouthful Trial a range of tastes and temperatures
Spitting out food and fluids	Offer a variety of food and drink Take note of food and drink that is accepted, and offer readily Liaise with the dietitian regarding supplements and offer intake in liquid form
Food/drink refusal	Try to stimulate appetite prior to meals by offering fruit juice and encouraging exercise Use indirect prompts e.g., "that looks nice" Encourage the person to try the first mouthful to "get a taste" Offer "grazing" meals or snacks throughout the day Attempt to offer the person familiar foods e.g., ask family to supply home cooked meals as able Ensure meal is high in calories/protein and liaise with dietitian Offer a range of options and cater to preferences Offer finger foods if appropriate Reduce distractions
Problems with teeth	If dentures have been left at home/care facility, organise someone to bring dentures to hospital. While waiting for dentures, offer a minced and moist diet. If the person usually eats without dentures, offer their usual diet. Use denture fixative for loose dentures Oral hygiene after all oral intake
Eating non-food items	Lock away all harmful or inappropriate items Ensure all involved are aware of the problem
Reduced level of consciousness	Only offer intake when the person is alert enough to swallow and able to maintain alertness for a sufficient amount of time Use a cold, wet face cloth to fully rouse the person before meals. Sit the person out of bed, or reposition in bed, to help them wake up.
Food residue in mouth post meals	Prompt the person to clear residue with finger or tongue Encourage a drink to aid oral clearance Massage cheeks to move residue centrally Mouth care at the end of each meal Upright positioning post meal for at least 30 minutes
Wandering during meal times	Use gentle physical prompts at the table e.g., put cup/cutlery back into the person's hands. Use simple verbal prompts and show the person their meal to aid understanding. Offer finger foods that can be consumed when "on the move" Gently guide the person back to the table when they wander, and prompt them to continue their meal. Reduce distractions

Dry mouth	Encourage regular sips of fluid during the day, particularly prior to meals. Swab the person's mouth with grape seed oil before meals Use an artificial saliva Offer extra sauce/gravy to moisten meal Encourage the person to alternate diet and fluids
Eating or drinking too slowly	Serve each course separately to retain warmth and appeal. If only small amounts are taken, liaise with the dietitian to ensure food is high in calories. Offer snacks between meals Provide full assistance if required
Eating too quickly/taking large mouthfuls	Minimise distractions and attempt to create a calm environment Serve courses separately Ensure food is chewed, swallowed and cleared prior to the next mouthful. Provide smaller/modified utensils e.g., teaspoon, spouted cup. Provide verbal and physical prompts to reduce rate of intake.
Eating from others' plates	Ensure individual boundaries are clear Use physical or verbal prompts to help the person identify their food and utensils. Supervise meals
Lack of initiative during meals	Draw the person's attention to their meal Describe what is on the plate Place cutlery into the person's hands Guide them to take the first mouthful Give verbal and physical prompts to continue their meal Sit the person with those more able, so they can be prompted by their example
Difficulty with utensils or messy eating	Cut food before serving Serve one course at a time Verbally orientate the person to the meal, plate and cutlery Place cutlery directly into the person's hands Refer to occupational therapist for modified utensils if required e.g., lipped plate, built-up cutlery, non-slip mat. Consider offering finger foods
Hemianopia/neglect	Place the meal on the person's good side Assist the person from their good side Turn the plate during the meal Provide verbal and physical prompts to attend to the neglected side during the meal
Pre-admission diet and fluids	Refer to nursing home/hostel transfer summary (if available) to determine pre-admission diet and fluids. If transfer summary is unavailable or unclear, contact the nursing home/hostel directly. If diet or fluids are modified, inform catering staff

Table reproduced from: from Ensell C, Matheson N. Mealtime behaviours in people with dementia in the absence of dysphagia: education of nursing staff in an acute care setting.

Appendix 5 Bringing Food from Home

Bringing food from home

Bringing in food from home is a lovely way to show a patient that you care for them. Whether it be culturally appropriate food, snacks, or their favourite family meal, it may help the patient to increase their food intake and lift their spirits.

If you do bring food from home, it is important that it has been prepared, transported, and stored safely to prevent food poisoning. Bacteria can grow when food is more than 5°C and less than 60°C.

When a person is in hospital their immune system is lowered, which puts them at higher risk of getting food poisoning.

This is a guide to assist you in identifying which foods are suitable for bringing in from home and how to make them as safe as possible.

At Armadale Kalamunda Group there are limited facilities to store patient food and food is not able to be reheated in microwaves.

If you are bringing food from home, we ask you to follow a few simple guidelines to reduce the risk of food poisoning:

Speak to the nurse

Patients may have special dietary requirements or restrictions while they are in hospital, so it is important that you check with your patient's nurse, dietitian or speech pathologist prior to giving them food or drinks.

Important: please do not offer or share food with other patients as they may have special dietary requirements or restrictions which you are unaware of.

Preparing food

Safe food preparation is important. The following tips will help ensure food is safely prepared.

- Wash your hands well with soap and water and dry thoroughly before handling food.
- Keep hot food steaming hot (60°C or above)
- Keep cold food refrigerated (between 0°C and 5°C)
- Separate raw and cooked/ready to eat food
- While cooking, ensure utensils and the kitchen are kept clean.
- If you are sick don't prepare food for patients.

Transporting food to the hospital

When transporting food to the hospital, it should be kept at the correct temperature to reduce the risk of harmful bacteria growth.

Cold food

- Maintain temperature under 5°C – refrigerator temperature.
- Put cold food in an insulated cooler bag.
- Pack food with an ice pack and do not pack hot and cold food together.

Hot food

- Maintain temperature above 60°C – steaming hot.
- Place in an insulated cooler bag with a hot water bottle.

Important: reheated food which has not been eaten cannot be refrigerated and should be thrown away.

Storing Food

To reduce the risk of food poisoning, food brought into the hospital needs to be stored correctly.

Please ask the ward coordinator if it is possible to store food in the ward's fridge as not all wards have the appropriate facilities for safe food storage.

Refrigerated food:

Any high risk or perishable food which is not eaten immediately needs to be covered and stored in a refrigerator. When storing food at the hospital:

- You may only store a small amount of food in the ward fridges.
- All stored food must be labelled with the patient's name, the date the food was prepared and the date it was put in the fridge.
- All potentially unsafe food stored in ward fridges and not consumed within 24 hours will be disposed of.

Non-refrigerated food:

Wrapped, non-perishable foods can all be stored at room temperature. These include:

- Biscuits
- Chocolates/lollies
- Crackers

Always check the 'Use by' or 'Best before' date on packaged, commercially prepared food.

Important: once packaging is opened it shortens the manufacturers 'Use by' date.

Patients with swallowing difficulties

Some patients may experience swallowing difficulties when they are in hospital. Swallowing is assessed by a speech pathologist, who may recommend the patient follow a texture modified diet or drink thickened fluids.

Texture modified diets:

Texture modified diets reduce the amount of chewing required, help patients swallow safely, and reduce the risk of food going down the wrong way.

There are three types of texture modified diets available:

- Smooth puree
- Minced and moist
- Soft

Thickened fluids:

Some patients may find it difficult to swallow thin fluids such as water, milk, tea, coffee etc. Thickened drinks may be safer because they move around the mouth and through the throat slower and allow patients better control during swallowing.

If your loved one has been recommended thickened fluids or a texture modified diet, it is important that you check with the patient's speech pathologist before bringing any food or drinks from home.

The table below gives specific information about types of food and the precautions required:

Food Type	Precautions
Shelf stable food including: • Cake • Chocolates • Cordial • Crisps • Biscuits • Fruit • Lollies • Soft drink	• Food can be stored in the patient's room • Ensure that packet food is used before the Use By date on the packet • Discard any food that looks spoiled
Cold perishable food including: • Custard • Cream filled cakes and biscuits • Hard cheese (e.g. cheddar) • Milk Drinks • Home prepared salads • Fruit salad/cut up fruit • Yoghurt	• Ensure foods such as fruit and salad are washed thoroughly before preparing • Ensure packet foods are within their Use By date. • Keep food covered during transportation to avoid contamination • Transport these foods cold in an insulated bag or esky (consider placing an icepack in the bag or esky to keep cold) • Ensure the food is eaten immediately • Discard any food that is not eaten within 30 minutes
Hot food from home or outside supplier (e.g. café or take-away)	• Only bring in food that has been freshly prepared that day • Store the food in the refrigerator at home if you are not bringing it in immediately after it is cooked • Ensure that food is only reheated once • If reheating the food at home, ensure that it is reheated to at least 75°C for 2 minutes to kill any bacteria in the food.

- | | |
|--|--|
| | <ul style="list-style-type: none"> • Store the food in an insulated container (away from cold food) to keep it as hot as possible during transportation • Food must be eaten within 2 hours of heating. • Discard food that has been left at room temperature for 2 hours or more |
|--|--|

Higher Risk Foods

There are some foods that have a higher risk of listeria infection. We ask that you do not bring these foods into the hospital for patients. This is important for ALL patients and especially important for pregnant women and patients with low immunity.

These include:

- Cold meats e.g. ham, cold cooked chicken, cold roast beef, pate
- Prepared salads e.g. from a deli or salad bar
- Chilled seafood e.g. sushi, sashimi, smoked salmon, prawns, oysters
- Soft cheese e.g. brie, camembert, ricotta, feta, blue cheese
- Soft serve dairy products
- Unpasteurised dairy products e.g. raw goat/cow milk/cheese/yoghurt
- Raw or undercooked egg e.g. egg nog, whole egg mayonnaise